Scenario 1

- Medicine Name: Metformin (Antidiabetic)
- Dose: 500 mg tablet, twice daily.
- Timeline: Started on September 1, 2026. Stopped on September 5, 2026.
- Adverse reaction: The patient developed mild nausea and diarrhea. The symptoms resolved after stopping the medicine. No hospital visit was required. These are known side effects of metformin.

Scenario 2

- Medicine Name: Warfarin (Anticoagulant)
- Dose: 5 mg tablet, once daily.
- Timeline: Started on August 15, 2026. Stopped on September 5, 2026.
- Adverse reaction: The patient developed severe gastrointestinal bleeding and was admitted to the hospital for three nights. Bleeding is a known risk of warfarin.

Scenario 3

- Medicine Name: Allopurinol (Antigout Medicine)
- Dose: 100 mg tablet, once daily.
- Timeline: Started on September 1, 2026. Stopped on September 10, 2026.
- Adverse reaction: The patient developed an unusual change in nail colour without other symptoms or a need for medical treatment. This reaction is assumed not to be listed in the product information

Scenario 4

- Medicine Name: Enalapril (Antihypertensive)
- Dose: 5 mg tablet, once daily.
- Timeline: Started on July 10, 2026. Treatment is still ongoing.
- Adverse reaction: The patient developed a persistent, dry cough that was uncomfortable but did not require hospitalisation. Dry cough is a known adverse effect of enalapril.

Scenario 5

- Medicine Name: Atorvastatin (Cholesterol-Lowering Medicine)
- Dose: 20 mg tablet, once daily.
- Timeline: Started on August 20, 2026. Stopped on September 2, 2026.
- Adverse reaction: The patient developed severe muscle pain and weakness with markedly elevated creatine kinase levels and was admitted to the ICU for rhabdomyolysis. Severe muscle injury is a recognised risk of statins.

Scenario 6

- Medicine Name: Carbamazepine (Antiepileptic)
- Dose: 200 mg tablet, twice daily.
- Timeline: Started on September 4, 2026. Stopped on September 12, 2026.
- Adverse reaction: The patient developed a severe blistering skin reaction diagnosed as Stevens–Johnson syndrome and required hospital admission. This is a recognised, rare adverse reaction to carbamazepine.

Scenario 7

- Medicine Name: Omeprazole (Proton Pump Inhibitor)
- Dose: 20 mg capsule, taken once daily.
- Timeline: Started on September 1, 2026. Treatment is still ongoing.
- Adverse reaction: The patient noticed a mild change in taste that did not interfere with eating or drinking and did not require medical treatment. For this hypothetical scenario, the reaction is assumed not to be listed in the applicable product information.

CLASSIFICATION OF SERIOUSNESS AND EXPECTEDNESS

Serial number	Medicine name	Adverse reaction	Seriousness	Expectedness	Rationale
1.	Metformin	Mild Nausea and diarrhea	Non-Serious	Expected	Diarrhea is a known adverse effect. No hospitalization or significant medical intervention is required in this scenario
2	Warfarin	Severe GI bleeding	Serious	Expected	Hospitalization makes the event serious. Bleeding is a known risk of

					anticoagulant therapy.
3	Allopurinol	Nail colour change	Non-serious	Non-serious	This potentially life-threatening skin reaction requires urgent hospital care. It is a recognized, rare adverse reaction to allopurinol.
4	Enalapril	Persistent dry cough	Non-Serious	Expected	Dry cough is a recognized adverse effect of ACE inhibitors and usually does not require hospitalization.
5	Omeprazole	Change in taste	Non-serious	Unexpected	Muscle pain is a known adverse effect of statins. In this scenario, it does not cause serious complications.

6	Atorvastatin	Rhabdomyolysis	Serious	Expected	This potentially life-threatening reaction requires urgent medical treatment. It is a recognized serious adverse effect of carbamazepine.
7	Carbamazepine	Stevens–Johnson syndrome	Serious	Expected	Headache is a recognized adverse effect and generally does not require hospitalization